

COMMUNITY-LED CREATIVE RESEARCH

Shoulder & Hip Advertising Angles

A simple report summarising public patient-community patterns, 20 testable creative angles, and the hypothesis behind each one.

Prepared for Heart Health Co.
Public-community research only

[Open shoulder product page >](#)

[Open hip product page >](#)

REPORT SUMMARY

What the community research is telling us

People rarely describe shoulder or hip discomfort with medical language. They describe the normal things it takes away: sleep, dressing, driving, travel, work, independence, and the ability to move without planning every step.

01

Specific moments outperform generic pain claims

The strongest ideas are concrete scenes people recognize immediately, such as waking at 2:00 a.m., fastening a bra, taking the first steps after sitting, or struggling with socks.

02

Function is the emotional endpoint

The audience wants ordinary independence: sleeping, dressing, driving, shopping, travelling, and reaching naturally.

03

Routine creates a natural product role

A wearable, hands-free home routine fits best before bed, after sitting or driving, after activity, and during the waiting period between appointments.

04

Validation matters

People who feel confused by scans, temporary relief, or recurring flares respond to language that acknowledges their experience without diagnosing them.

Six angles to test first

- The 2:00 a.m. shoulder
- The bra and hair milestone
- The first three steps after sitting
- The side-sleeper's hip routine
- Heat and massage helped, then it returned
- When the scan does not explain the discomfort

Recommended first test: Keep the offer, audience, and landing page constant. Create one 9-15 second hook, one 30-45 second UGC, one static image, and one advertorial headline for each concept. Judge the angle using hook rate, CTR, landing-page views, add-to-cart rate, and conversion rate.

Research scope

The review used public Facebook groups and additional patient communities. No private groups were joined and no personal information was collected.

[Shoulder product page >](#)

[Hip product page >](#)

SHOULDER ANGLES

Angles 1-2

01 The 2:00 a.m. shoulder

Hook: "Why shoulder discomfort often feels louder after midnight."

Story: During the day, the person can manage the discomfort by staying busy. At night, every position feels wrong. Sleeping on the affected side hurts, the good side pulls on the shoulder and lying on the back creates pressure around the shoulder blade. The person gets up for ice, changes pillows and wakes exhausted. Introduce the Heart Health brace as a convenient 20-minute comfort routine before bed.

Why it may work: Sleep disruption creates more urgency than general pain relief. It is repeated every night, emotionally exhausting and closely connected to the product's hands-free use. Night pain and complicated pillow arrangements are among the most common community discussions. Community sleep discussion

Best formats: 30-second nighttime UGC, dark-bedroom static and a longer bedtime-routine advertorial.

[Source: Community sleep discussion >](#)

02 The bra and hair milestone

Hook: "The recovery milestone nobody puts on a medical chart."

Story: The problem begins as a small ache. Gradually, fastening a bra, brushing hair, applying deodorant and pulling on a jacket become difficult. The desired outcome is not athletic performance. It is being able to get dressed independently again. Demonstrate the product as part of a consistent daily routine, then show function-based progress.

Why it may work: Specific activities produce stronger self-recognition than "shoulder pain." The bra and hair examples are especially relevant to women aged 45 and older, which closely matches Heart Health's likely ICP. A recent community post celebrating the ability to fasten a bra received strong positive engagement. Bra milestone discussion

Best formats: Female UGC diary, before-and-after movement demonstration and static checklist.

[Source: Bra milestone discussion >](#)

SHOULDER ANGLES

Angles 3-4

03 The wrong-movement "zinger"

Hook: "It feels manageable until one ordinary movement catches you by surprise."

Story: The shoulder carries a dull ache during normal activity. Then the person reaches into the back seat, pulls a blanket, puts on a jacket or reaches behind the body. A sudden sharp pain stops the movement. The story explains that unpredictable movements create fear and make people consciously guard the shoulder. Introduce a consistent home routine focused on comfort and supporting everyday movement.

Why it may work: The unexpected sharp movement is highly recognizable and visually dramatic. It creates a strong video pattern interrupt without needing an exaggerated medical claim. Community members frequently describe these sudden pains during ordinary, unplanned movements. Shoulder-stage discussion

Best formats: Fast montage of three accidental movements followed by a product demonstration.

[Source: Shoulder-stage discussion >](#)

04 The pain improved, but movement did not

Hook: "Less pain does not always mean normal movement has returned."

Story: The constant ache begins to decrease, so the person assumes the shoulder is recovering. However, reaching behind the back, lifting the arm and doing the hair remain difficult. This creates confusion about whether the condition is improving. Present Heart Health as a consistent comfort and mobility-support routine, with progress measured through everyday movements rather than pain alone.

Why it may work: This speaks to people who are partially improved but still frustrated. They are more solution-aware and may be ready to try something new because their original pain solution did not restore function.

Best formats: Range-of-motion diary, educational animation and customer story.

Source context: Shoulder community patterns summarised in the research.

SHOULDER ANGLES

Angles 5-6

05 Life becomes smaller one activity at a time

Hook: "Pain rarely takes everything at once. It takes one normal thing at a time."

Story: First, the person stops sleeping on one side. Then carrying groceries, preparing food, driving, showering and hobbies become harder. Eventually, daily life revolves around avoiding certain movements. The story shifts the endpoint from "reduce pain" to "protect the life and independence you still have."

Why it may work: Loss aversion is more powerful than a generic relief promise. The public Facebook shoulder group repeatedly frames the problem as life becoming smaller and normal activities no longer feeling easy. Facebook shoulder support group

Best formats: Emotional long-form ad and day-in-the-life UGC.

[Source: Facebook shoulder support group >](#)

06 When it is your dominant arm and you live alone

Hook: "When the painful shoulder is the arm you rely on for everything."

Story: The person switches to a front-clasp bra, spray deodorant, pre-cut vegetables and a different computer mouse. Every workaround costs time, money or independence. Demonstrate how a hands-free wearable routine can fit into the day without requiring someone else to hold a device or repeatedly apply creams.

Why it may work: Independence is a powerful emotional endpoint. The product's wearable design is particularly relevant to people who live alone or cannot rely on daily assistance. These exact workarounds appear repeatedly in frozen-shoulder discussions. Day-to-day coping discussion

Best formats: Day-in-the-life UGC showing five frustrating workarounds.

[Source: Day-to-day coping discussion >](#)

SHOULDER ANGLES

Angles 7-8

07 The compensation chain

Hook: "When the neck and shoulder blade begin doing the shoulder's job."

Story: Because certain shoulder movements feel uncomfortable, the person changes how they reach, sleep and work. Other areas start feeling tight or overworked. The story explains that guarding one area can change how the rest of the upper body moves. Position the product as a targeted shoulder routine rather than another general back or neck product.

Why it may work: This provides a new mechanism-based explanation without requiring a specific diagnosis. It also expands the emotional cost beyond one isolated joint. Community members frequently mention neck, shoulder-blade and back discomfort from compensating.

Best formats: Clinician-style animation or creator demonstration.

Source context: Shoulder community patterns summarised in the research.

08 The waiting-list gap

Hook: "The difficult week between the scan and the next appointment."

Story: The person is in significant discomfort but cannot see a specialist for several days or weeks. They are unsure what the scan means and worried about making the problem worse. Present the product as a conservative at-home comfort option while they continue seeking professional advice, not as a substitute for medical evaluation.

Why it may work: Waiting creates high intent, uncertainty and a need for something safe and practical. A public Facebook post asked for help while waiting for a shoulder appointment, and community members frequently discuss month-long waits for injections or follow-ups. Facebook shoulder support group

Best formats: Educational advertorial with clear safety guidance and red-flag instructions.

Source: [Facebook shoulder support group >](#)

SHOULDER ANGLES

Angles 9-10

09 Heat helped. Massage helped. Then it returned.

Hook: "Why single-mode relief can feel incomplete."

Story: The person has tried heating pads, massage, creams and exercises. Each may feel helpful in the moment, but managing several separate treatments becomes inconvenient. Introduce Heart Health's four-mode wearable as one routine combining red light, near-infrared light, controlled heat and vibration.

Why it may work: This explains the product through convenience and integration rather than claiming every previous treatment failed. It is best for a skeptical, solution-aware customer who understands the category but has not found a routine they can maintain.

Best formats: Four-way product demonstration, comparison static and longer mechanism VSL.

Source context: Shoulder community patterns summarised in the research.

10 Reaching without thinking first

Hook: "The real win is reaching without checking your shoulder first."

Story: The desired outcome is not a dramatic transformation. It is pulling on a seatbelt, reaching for a cup, grabbing something from the back seat or putting on a jacket without planning the movement. Follow a real customer's function-based progress over 30 days.

Why it may work: This is believable, emotional and easy to demonstrate. It replaces an abstract pain score with an automatic behavior people deeply miss.

Best formats: Thirty-day UGC diary and customer testimonial.

Source context: Shoulder community patterns summarised in the research.

HIP ANGLES

Angles 11-12

11 The first three steps after sitting

Hook: "What the first three steps after sitting can reveal about hip discomfort."

Story: The person feels reasonably comfortable while seated. When they stand, the first few steps feel stiff, deep or guarded. After moving for a while, it sometimes eases. The story introduces a post-sitting home routine and explains why consistent support may matter more than waiting for the next flare.

Why it may work: This is highly visual, easy to demonstrate and happens several times a day. Deep-buttock discomfort after prolonged sitting is one of the clearest Facebook hip-community patterns. Sciatica / Hips & Back Pain Central

Best formats: Chair-test video, symptom-pattern static and advertorial.

Source: [Sciatica / Hips & Back Pain Central >](#)

12 The 30-minute drive

Hook: "Why a short drive can feel harder than a long walk."

Story: The person manages walking reasonably well, but office chairs and car seats create deep discomfort around the hip or groin. They arrive home and have to pause before getting out of the car. Introduce a portable routine that can be used after work or travel.

Why it may work: Driving and desk work are high-frequency triggers that competitors do not own strongly. People with hip impingement repeatedly identify sitting and driving as their biggest everyday problems. Sitting and driving discussion

Best formats: Office-to-car UGC and "three chairs that trigger discomfort" static.

Source: [Sitting and driving discussion >](#)

HIP ANGLES

Angles 13-14

13 The side-sleeper's impossible position

Hook: "The bedtime pillow arrangement nobody understands until hip discomfort begins."

Story: Sleeping on the affected side creates pressure. Sleeping on the other side may pull across the hip. Sleeping on the back feels unnatural. The person begins arranging pillows between the knees and around the body. Introduce a short pre-bed comfort routine that complements their existing sleep setup.

Why it may work: As with shoulder discomfort, sleep creates immediate urgency and a repeatable product ritual. Nighttime hip pain and side-sleeping difficulty appear throughout hip-pain communities. Hip night-pain discussion

Best formats: Bedroom UGC and pillow-position comparison static.

[Source: Hip night-pain discussion >](#)

14 The holiday pain tax

Hook: "The part of the holiday photos never show."

Story: The person completes the flight, drives to the beach and walks around sightseeing. They look fine in photos, but by evening the hip is throbbing and the next day's plans depend on how it feels. Position the product as a portable recovery routine for active days and travel.

Why it may work: This sells the ability to participate in life rather than simply promising relief. It is aspirational, emotional and particularly relevant to active adults who do not identify as disabled. Holiday flare discussion

Best formats: Travel UGC, packing demonstration and creator diary.

[Source: Holiday flare discussion >](#)

HIP ANGLES

Angles 15-16

15 The good-day flare

Hook: "A good day can become tomorrow's flare."

Story: The person has a better day and walks farther, exercises harder or completes more chores. That evening or the following morning, the discomfort returns. They begin worrying that every activity will create another setback. Present the product as a consistent post-activity routine, supported by a simple activity and comfort diary.

Why it may work: The boom-and-bust cycle is common in chronic pain communities. It creates a recurring use case and supports retention because the product is positioned as a routine, not a one-time rescue.

Best formats: Two-day UGC sequence: good day versus next-morning flare.

Source context: Hip community patterns summarised in the research.

16 When socks become a 10-minute task

Hook: "The small morning task that reveals how much hip movement has changed."

Story: Putting on socks, stepping into trousers, tying shoes and getting into the car begin taking longer. The person may need a chair, grabber or another person's help. The desired endpoint is simple independence with dressing and leaving the house.

Why it may work: Like the shoulder bra angle, socks and shoes are concrete and instantly recognizable. Dressing difficulties generate substantial community discussion because they expose the practical cost of reduced hip movement. Getting-dressed discussion

Best formats: Morning-routine UGC and mobility diary.

Source: [Getting-dressed discussion >](#)

HIP ANGLES

Angles 17-18

17 When the scan does not explain the discomfort

Hook: "When the scan says little, but sitting and side-sleeping still hurt."

Story: The person receives an X-ray or initial consultation but still lacks a clear answer. The discomfort remains during sitting, driving or lying on the affected side. The story explains that hip-region discomfort can involve different structures and that proper assessment still matters. Present Heart Health as non-invasive comfort support while the person continues working with a clinician.

Why it may work: People who feel dismissed or confused respond strongly to validation. It also creates a mechanism-open customer who wants to understand soft-tissue support, without the ad diagnosing them. No-answers hip discussion

Best formats: Clinician-led advertorial and "scan versus daily experience" static.

[Source: No-answers hip discussion >](#)

18 Not ready to make a surgical decision

Hook: "A non-invasive comfort routine while you work through your options."

Story: The person is afraid of surgery, the recovery period, needing a walker and relying on others. They are not necessarily rejecting medical care. They want a conservative option they can use while discussing treatment choices with their clinician. Present the belt as supportive home care, not as a replacement for surgery.

Why it may work: This reaches a highly motivated, solution-aware audience while remaining safer than "avoid replacement" messaging. Fear, limited support and recovery uncertainty are prominent in both Facebook and Reddit hip communities. Surgery-fear discussion

Best formats: Long-form founder or clinician video with restrained claims.

[Source: Surgery-fear discussion >](#)

HIP ANGLES

Angles 19-20

19 Mini goals, not miracles

Hook: "The first grocery trip can matter more than a pain score."

Story: Instead of presenting an instant transformation, follow progress through small goals: getting out of bed, shopping, returning to desk work, walking the dog and sleeping more comfortably. The product becomes part of a measured daily routine.

Why it may work: Small wins are believable and lower customer skepticism. Facebook hip-recovery discussions repeatedly celebrate grocery shopping, returning to work, sleeping and other practical milestones. Australian Hip Replacement Support Group

Best formats: Thirty-day diary, customer carousel and weekly check-in UGC.

Source: [Australian Hip Replacement Support Group >](#)

20 "I thought I was finally in the clear"

Hook: "When discomfort returns after a good stretch."

Story: The person experiences several good weeks and assumes the problem has settled. Then sitting, travel or increased activity causes another flare. The story reframes the product as a consistent maintenance routine rather than something used only when the discomfort becomes severe.

Why it may work: Fear of recurrence creates a strong reason for routine use and repeat engagement. This angle may also improve customer retention because the product is associated with consistency rather than emergency use.

Best formats: Customer story, timeline carousel and post-purchase education.

Source context: Hip community patterns summarised in the research.

TESTING AND LINKS

Recommended testing plan

Testing order

1. Shoulder night routine
2. Bra and hair milestone
3. Hip chair test
4. Hip side-sleeping routine
5. Single-mode versus four-mode routine
6. Scan mismatch and soft-tissue education

Deliverables for each concept

- One 9-15 second hook video
- One 30-45 second UGC
- One static image
- One advertorial headline variation

Advertising safeguard

Use educational language, such as 'Why shoulder discomfort often feels worse at night.' Do not diagnose the viewer, guarantee treatment, or position the product as a substitute for medical evaluation.

Key community links

[Shoulder Pain Solution\(s\) + Support\(s\) >](#)

[Sciatica / Hips & Back Pain Central >](#)

[Australian Hip Replacement Support Group >](#)

[Frozen Shoulder community >](#)

[Hip Impingement community >](#)

[Meta: health-information protections >](#)

[Meta: audience guidance >](#)

[Shoulder product page >](#)

[Hip product page >](#)